Atypical responses (i.e., an initial transient increase in tumour size or small new lesions within the first few months followed by tumour shrinkage) have been observed. It is recommended to continue treatment with nivolumab or nivolumab in combination with ipilimumab for clinically stable patients with initial evidence of disease progression until disease progression is confirmed.

Dose escalation or reduction is not recommended. Dosing delay or discontinuation may be required based on individual safety and tolerability. Guidelines for permanent discontinuation or withholding of doses are described in Table 4. Detailed guidelines for the management of immune-related adverse reactions are described in section 4.4.

Table 4: Recommended treatment modifications for OPDIVO or OPDIVO in combination with ipilimumab

Immune-related adverse reaction	Severity	Treatment modification
Immune-related pneumonitis	Grade 2 pneumonitis	Withhold dose(s) until symptoms resolve, radiographic abnormalities improve, and management with corticosteroids is complete
	Grade 3 or 4 pneumonitis	Permanently discontinue treatment
Immune-related colitis	Grade 2 diarrhoea or colitis	Withhold dose(s) until symptoms resolve and management with corticosteroids, if needed, is complete
	Grade 3 diarrhoea or colitis - OPDIVO monotherapy	Withhold dose(s) until symptoms resolve and management with corticosteroids is complete
	- OPDIVO+ipilimumab[a]	Permanently discontinue treatment
	Grade 4 diarrhoea or colitis	Permanently discontinue treatment
Immune-related hepatitis	Grade 2 elevation in aspartate aminotransferase (AST), alanine aminotransferase (ALT), or total bilirubin	Withhold dose(s) until laboratory values return to baseline and management with corticosteroids, if needed, is complete
	Grade 3 or 4 elevation in AST, ALT, or total bilirubin	Permanently discontinue treatment
Immune-related nephritis and renal dysfunction	Grade 2 or 3 creatinine elevation	Withhold dose(s) until creatinine returns to baseline and management with corticosteroids is complete
	Grade 4 creatinine elevation	Permanently discontinue treatment
Immune-related endocrinopathies	Symptomatic Grade 2 or 3 hypothyroidism, hyperthyroidism, hypophysitis, Grade 2 adrenal insufficiency Grade 3 diabetes	Withhold dose(s) until symptoms resolve and management with corticosteroids (if needed for symptoms of acute inflammation) is complete. Treatment should be continued in the presence of hormone replacement therapy[b] as long as no symptoms are present
	Grade 4 hypothyroidism Grade 4 hyperthyroidism Grade 4 hypophysitis Grade 3 or 4 adrenal insufficiency Grade 4 diabetes	Permanently discontinue treatment
Immune-related skin adverse reactions	Grade 3 rash	Withhold dose(s) until symptoms resolve and management with corticosteroids is complete

Table 4: Recommended treatment modifications for OPDIVO or OPDIVO in combination with ipilimumab

Immune-related adverse reaction	Severity	Treatment modification
	Grade 4 rash	Permanently discontinue treatment
	Stevens-Johnson syndrome (SJS) or toxic epidermal necrolysis (TEN)	Permanently discontinue treatment (see section 4.4)
Immune-related myocarditis	Grade 2 myocarditis	Withhold dose(s) until symptoms resolve and management with corticosteroids is complete[c]
	Grade 3 or 4 myocarditis	Permanently discontinue treatment
Other immune-related adverse reactions	Grade 3 (first occurrence)	Withhold dose(s)
	Grade 4 or recurrent Grade 3 ; persistent Grade 2 or 3 despite treatment modification; inability to reduce corticosteroid dose to 10 mg prednisone or equivalent per day	Permanently discontinue treatment

Note: Toxicity grades are in accordance with National Cancer Institute Common Terminology Criteria for Adverse Events Version 4.0 (NCI-CTCAE v4).

[a] During administration of the second phase of treatment (nivolumab monotherapy) following combination treatment, permanently discontinue treatment if Grade 3 diarrhoea or colitis occurs.

[b] Recommendation for the use of hormone replacement therapy is provided in section 4.4.

[c] The safety of re-initiating nivolumab or nivolumab in combination with ipilimumab therapy in patients previously experiencing immune-related myocarditis is not known.

OPDIVO or OPDIVO in combination with ipilimumab should be permanently discontinued for:

- Grade 4 or recurrent Grade 3 adverse reactions;
- Persistent Grade 2 or 3 adverse reactions despite management.

Patients treated with OPDIVO must be given the patient alert card and be informed about the risks of OPDIVO (see also package leaflet).

When OPDIVO is administered in combination with ipilimumab, if either agent is withheld, the other agent should also be withheld. If dosing is resumed after a delay, either the combination treatment or OPDIVO monotherapy could be resumed based on the evaluation of the individual patient.

Special populations

Paediatric population
The safety and efficacy of OPDIVO in children below 18 years of age have not been established. No data are available.

Elderly
No dose adjustment is required for elderly patients (≥ 65 years) (see section 5.2).
Data from SCCHN, adjuvant melanoma and first-line RCC patients 75 years of age or older are too limited to draw conclusions on this population (see sections 4.8 and 5.1).

Renal impairment
Based on the population pharmacokinetic (PK) results, no dose adjustment is required in patients with mild or moderate renal impairment (see section 5.2). Data from patients with severe renal impairment are too limited to draw conclusions on this population.

Hepatic impairment
Based on the population PK results, no dose adjustment is required in patients with mild hepatic impairment (see section 5.2). Data from patients with moderate or severe hepatic impairment are too